

This Patient Group Direction (PGD) is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.

Patient Group Direction

Shingles (Herpes Zoster) Treatment

Aciclovir, Valaciclovir or Famciclovir

New PGD. Replaces a slug that was serving the Shingrix vaccine document.

This PGD is for the TREATMENT of active shingles with oral antivirals. It is not about vaccination. The catalogue slug for shingles treatment was previously serving the Shingrix vaccine PGD, which contains no antiviral at all, so a pharmacist opening what they believed was the treatment document found a vaccination document instead. That is the gap this PGD fills.

Note on NHS Pharmacy First. Shingles is a commissioned Pharmacy First pathway in England with national PGDs for aciclovir and valaciclovir. Where a patient is eligible for that service it should be used, and this PGD should not displace it. This PGD exists for private supply, for patients outside NHS eligibility, and to cover famciclovir, which the national service does not include.

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	• Pharmacist registered and practicing with the GPhC • Pharmacy technician registered and practicing with the GPhC
Training and assessment	Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. • All users must be familiar with the SPC for the products supplied, as well as BNF advice and any applicable national guidance • All users must be competent in the recognition of herpes zoster and, critically, in distinguishing it from the presentations that require urgent referral rather than supply • All users must be able to assess pain using a validated scale and to recognise the features of ophthalmic involvement, including Hutchinson's sign • All users must be familiar with the definition of severe immunosuppression in Green Book chapter 28a • All users must previously have used PGD's to supply or administer medication • Must work in compliance with the SOPs of their own employer • All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	• All users must be up to date with CPD requirements and appraisal • All users must be up to date with MHRA and safety alerts with regards to medicinal products • All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005

Clinical condition or situation to which this PGD applies

PGD Indication	Treatment of uncomplicated herpes zoster (shingles) in immunocompetent adults, to reduce the severity and duration of the episode and to reduce the risk of post-herpetic neuralgia. Shingles results from reactivation of latent varicella zoster virus and presents as a painful
----------------	--

	vesicular rash in a dermatomal distribution that does not cross the midline.
Inclusion criteria	• Adults aged 18 years and over. • A clinical presentation consistent with shingles: a unilateral, dermatomal, painful vesicular rash that does not cross the midline, with or without a preceding prodrome of pain, itching or tingling. • Rash onset within 72 hours, AND at least one of: age over 50; non-truncal involvement of the neck, limbs or perineum; moderate or severe pain; or moderate or severe rash with confluent lesions. • OR rash onset within 7 days, AND at least one of: continued formation of new vesicles; severe pain; age 70 or over; or a high risk of severe shingles, for example severe atopic eczema. • Valid informed consent obtained. • No exclusion criteria or red flags present.
Red flags requiring urgent referral rather than supply	• Ophthalmic involvement. Rash in the distribution of the ophthalmic division of the trigeminal nerve, any visual symptom, an unexplained red eye, or Hutchinson's sign, which is a rash on the tip, side or root of the nose and predicts subsequent eye inflammation. Refer the same day for ophthalmology assessment. • Ramsay Hunt syndrome or facial nerve involvement. Rash in or around the ear, hearing loss, vertigo, altered taste, or unilateral facial weakness. Refer urgently. • Signs of meningitis: neck stiffness, photophobia, mottled skin. Refer to A&E. • Signs of encephalitis: disorientation, confusion, change in behaviour. Refer to A&E. • Signs of myelitis: muscle weakness, loss of bladder or bowel control. Refer to A&E. • Disseminated or widespread rash, or rash crossing the midline, which suggests dissemination. • Severe immunosuppression, as defined in Green Book chapter 28a. Refer to A&E if the rash is widespread or severe, or the patient is systemically unwell. • Any sign of sepsis or serious systemic infection. Call 999. • Pain not controlled by over-the-counter analgesia. Refer to a prescriber the same day. • Systemic illness not meeting the threshold for sepsis. Refer to a prescriber the same day.
Exclusion criteria	• Under 18 years of age. • Pregnancy, known or suspected. Refer to a prescriber. • Breastfeeding where there are sores on the breast. Sores elsewhere are a caution rather than an exclusion. • Rash onset more than 7 days ago. Refer for a prescriber decision, which may still favour treatment where new vesicles are forming or pain is severe. • Severe immunosuppression as defined in Green Book chapter 28a. See the red flags above. • Renal impairment below the thresholds in the dosing section. Refer rather than attempting to operate a renal dosing ladder in the pharmacy. • Known hypersensitivity to the antiviral to be supplied. Aciclovir

	and valaciclovir are cross-reactive; famciclovir and penciclovir are cross-reactive. - Any previous DRESS reaction to valaciclovir or famciclovir. These must never be restarted. - Concurrent ciclosporin, tacrolimus, mycophenolate, aminophylline or theophylline. Refer to a prescriber. - Inability to swallow or absorb oral medication. - Current long-term prophylactic treatment with the same class of antiviral. - Any underlying neurological condition. - Unable to maintain adequate fluid intake, or at risk of dehydration. - Failure to respond to antiviral treatment already given for this episode.
Cautions	- Elderly patients. Renal function declines with age, often without a formal diagnosis of chronic kidney disease, and all three antivirals carry a higher risk of neurological adverse effects in this group, including confusion, hallucinations and somnolence. Check renal function before supply. - Other nephrotoxic medicines, including ACE inhibitors, angiotensin receptor blockers, diuretics, NSAIDs, metformin, aminoglycosides and methotrexate. Supply may still be appropriate but counsel firmly on maintaining fluid intake. - Tenofovir. Supply may proceed, but advise the patient to contact the prescriber of their tenofovir about additional renal monitoring. - Probenecid or cimetidine. These reduce renal clearance of aciclovir and valaciclovir. The therapeutic index is wide so this is usually not significant, but it matters more where renal function is already reduced. - Raloxifene with famciclovir. Raloxifene inhibits the enzyme that converts famciclovir to its active form, so antiviral effect may be reduced. Monitor the clinical response. - Non-severe immunosuppression. Valaciclovir and famciclovir are both licensed for herpes zoster in immunocompromised adults; aciclovir is not licensed in the same terms. Where a patient has non-severe immunosuppression, use valaciclovir or famciclovir rather than aciclovir.
Actions if patient is excluded or declines treatment	- Discuss the reason for exclusion with the patient and ensure they understand it. - Advise on alternative options and how these can be accessed, for example the GP, a travel clinic, or a specialist service. - Document the reason for exclusion, the advice given and the decision reached. - Inform or refer to the GP as appropriate, and where the exclusion is time critical make the urgency of that referral explicit to the patient.

Details of the medicine

Name, form and strength of medicine	- Aciclovir 800 mg tablets or dispersible tablets. - Valaciclovir 500 mg film-coated tablets. - Famciclovir 250 mg or 500 mg film-coated tablets. - Choose one agent. Do not combine.
-------------------------------------	--

Legal category	POM
Storage	Store in the original container. Follow the storage conditions in the SPC for the product supplied.
Route/method of administration	Oral.
Choice of agent	• Aciclovir 800 mg five times daily is the cheapest option but requires five doses a day at roughly four hourly intervals, omitting the night dose. Adherence is the practical problem. • Valaciclovir 1000 mg three times daily is preferred where five times daily dosing is impractical: patients whose medicines are given by a carer, those already taking eight or more medicines a day, and those with non-severe immunosuppression. • Famciclovir 500 mg three times daily is an alternative with the same adherence advantage as valaciclovir. It is not part of the NHS Pharmacy First service, so it is most relevant to private supply.
Dose and frequency of administration	• Aciclovir: 800 mg five times daily at approximately four hourly intervals, omitting the night dose, for 7 days. • Valaciclovir: 1000 mg three times daily for 7 days. • Famciclovir, immunocompetent: 500 mg three times daily for 7 days. • Famciclovir, non-severe immunosuppression: 500 mg three times daily for 10 days. • Start treatment as soon as possible. The benefit falls away the longer the delay after rash onset.
Renal function thresholds	Do not attempt to operate the full renal dosing ladder in a community pharmacy. Where renal function falls below the thresholds below, refer to a prescriber. • Aciclovir: do not supply under this PGD where eGFR is below 30 mL/min/1.73m². • Valaciclovir: do not supply under this PGD where eGFR is below 60 mL/min/1.73m². This is deliberately more conservative than the aciclovir threshold and matches the national Pharmacy First PGD. • Famciclovir: do not supply under this PGD where eGFR is below 60 mL/min/1.73m², by analogy with valaciclovir. There is no national PGD for famciclovir to benchmark against, so this is a house position taken on the conservative side. • Where renal function is unknown and the patient is elderly or has risk factors for renal impairment, refer rather than assume.
Quantity to be administered and/or supplied	One complete course as specified above. No repeat supply under this PGD.
Adverse effects	Common to all three: headache, dizziness, nausea, vomiting, diarrhoea, abdominal pain, rash and pruritus. Aciclovir and valaciclovir can cause photosensitivity. Uncommon or rare across the class: confusion, hallucinations, agitation, tremor, ataxia, reduced consciousness, convulsions, encephalopathy and coma, most often in renal impairment or the elderly; acute renal failure; anaphylaxis; angioedema; blood dyscrasias. Valaciclovir and famciclovir have both been associated with DRESS, and famciclovir with serious skin reactions including Stevens-Johnson syndrome and toxic epidermal necrolysis. Consult the current SPC for the product supplied.
Yellow card reporting	Report suspected adverse reactions via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: • that valid informed consent was given

	• name of individual, address, date of birth and GP with whom the individual is registered • name of healthcare practitioner • the date of rash onset and which treatment window applies • the dermatome affected and the pain score recorded • that red flags were assessed and found absent, and that ophthalmic involvement was specifically excluded • renal function and how it was established • name and brand of medicine, dose, quantity and batch number • advice given, including advice given if excluded or if the patient declines • details of any adverse reaction and the action taken • that the medicine was supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Adults aged 18 and over: keep records for 8 years.
--	---

Pain, infection control and safety netting

Pain

- For mild pain, advise paracetamol alone or with codeine, or an NSAID such as ibuprofen, subject to the usual contraindications.
- Refer urgently to a prescriber if pain is not controlled by over-the-counter analgesia.
- Explain post-herpetic neuralgia: pain persisting after the rash heals, more common with increasing age. It is managed with neuropathic pain treatments that require a prescription, so the patient should see their GP if pain persists.

Infection control

- The patient can transmit chickenpox, not shingles, to someone who has never had chickenpox or the varicella vaccine. Transmission is by direct contact with vesicle fluid.
- They remain infectious until all the vesicles have crusted over, usually 5 to 7 days after the rash appears.
- Advise them specifically to avoid pregnant women who have not had chickenpox, babies under one month old, and anyone who is immunosuppressed.
- Practical advice: cover weeping lesions that are not under clothing, keep the rash clean and dry, wear loose clothing, wash hands often, do not share towels or clothes, and avoid topical creams and adhesive dressings.
- Work, school and childcare need only be avoided while the rash is weeping and cannot be covered.

Safety netting

- Seek medical advice if the shingles has not resolved within 4 weeks.
- Seek advice if symptoms worsen significantly at any point, or do not improve after finishing the course.
- Seek advice if new vesicles are still appearing after 7 days of treatment.
- Seek advice for any new eye symptom, however minor.
- Seek advice if pain is not controlled.
- Call 999 or go to A&E for signs of sepsis, confusion, neck stiffness, weakness or loss of bladder or bowel control.
- Maintain a good fluid intake throughout the course, particularly if elderly.
- Once recovered, discuss the shingles vaccine with the GP practice. This is separate from treatment and is covered by a different PGD.

Patient information

- Give the patient the manufacturer's patient information leaflet.

- Explain the dosing schedule clearly and, for aciclovir, that five doses a day is demanding and the course will not work well if doses are missed.
- Give the infection control and safety netting advice above and record that it was given.
- Advise that antivirals reduce the severity and duration of the episode but do not cure it instantly, and that some pain may persist.
- Advise the patient to return any unused medicine to a pharmacy for disposal.

References

- NICE Clinical Knowledge Summary. Shingles. Current version.
- NHS England. Pharmacy First service, shingles clinical pathway and the national PGDs for aciclovir and valaciclovir. Current versions.
- Summary of Product Characteristics for aciclovir, valaciclovir and famciclovir, current versions, electronic Medicines Compendium.
- UKHSA. Immunisation against infectious disease, the Green Book, chapter 28a, for the definition of severe immunosuppression.
- British National Formulary, current edition.

Agreement to practise by the Registered Healthcare Professional

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of pharmacy premises / clinic premises to which this PGD relates	

Name of healthcare professional	Job title	Registration number	Signature

Valid from date	Expiry date
25/8/26	31/7/27

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or vaccine.

To be completed by the adopting pharmacy: the first block is signed by that pharmacy's superintendent or clinical lead, and the second by its professional group signatory. Get Real Health signs the authorship block below, not these.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date

Name of professional group signatory	Job title & name of organisation	Signature	Date

Signed on behalf of Get Real Health

Name	Qualification	Signature	Date
Nitin Shori	Medical Doctor GMC: 6047293		21/08/2026
Chris Pilkington	Pharmacist GPhC: 2046322		21/08/2026

Change history

Version number	Change details	Date
001	New PGD. Created because the shingles treatment entry in the catalogue was serving the Shingrix vaccine document, which contains no antiviral. A pharmacist opening it expecting treatment guidance found a vaccination PGD. Found on 21 August 2026 during a fingerprint of every document in the catalogue, prompted by the Pharmacy Plus Health query about the combined travel PGD. Renal thresholds follow the national Pharmacy First PGDs rather than the full SPC dosing ladders, on the basis that titrating renal doses is a prescriber task. The famciclovir threshold is a house position by analogy, as no national PGD exists for it.	21st August 2026